# 1. Four principles applied

## Principles

- **Beneficence**: act in the patient's best interests (realistic benefit)
- **Non-maleficence**: avoid burdensome treatment without benefit (CPR, ECMO in MOF + dementia)
- **Autonomy**: patient's own values via advance directive or surrogate (substituted judgement); families cannot demand inappropriate treatment
- **Justice**: fair use of ICU resources

## In the "do everything" case

- Respect family role but decision is about the patient's values, not family wishes
- Explore what "everything" means: fear, guilt, religion, mistrust

**Key point:** Apply each principle to the scenario, not as a list of definitions.

# 2. Structuring a family conference

### Before

- Quiet room, key family, nurse, senior doctor, interpreter, consistent team message, review facts

### During (VALUE / SPIKES)

- **V**alue what family say
- **A**cknowledge emotions
- **L**isten
- **U**nderstand the patient as a person
- **E**licit questions
- Ask what they know, give a warning shot, explain prognosis plainly, silence, recommendation

### After

- Document, follow-up meeting, support (chaplain, social worker)

### Evidence

- Lautrette 2007: VALUE + brochure ↓ PTSD, anxiety, depression in relatives

**Key point:** Make a recommendation; do not hand the decision burden to the family.

# 3. Futility, PIT and time-limited trial

## Definitions (ATS 2015)

- **Futile**: cannot achieve its physiological goal (strict, rare; e.g. CPR in refractory MOF)
- **Potentially inappropriate treatment**: some chance of effect but competing ethical considerations (burden vs benefit)
- **Time-limited trial**: agreed period (e.g. 3–7 days) with defined goals, then reassess

## Resolving conflict

- Communication, second opinion, ethics committee, mediation, offer transfer, legal route last

**Key point:** Use "potentially inappropriate", not "futile", unless strictly physiological.

# 4. Withdrawing life-sustaining treatment

## Process

- Consensus and documentation; DNACPR
- Withdrawing and withholding are ethically equivalent
- Stop non-comfort measures (bloods, monitoring alarms, feeds as appropriate)

## Practical

- Symptom control: opioids and benzodiazepines titrated to distress (double effect; not to hasten death)
- Vasopressors stopped; terminal extubation or terminal wean; anticipate secretions (glycopyrronium)
- Family presence, privacy, spiritual support, organ donation referral (DCD) before withdrawal

**Key point:** Comfort is the goal; document intent and doses.

# 5. Capacity, consent and law

### Capacity

- Understand, retain, weigh, communicate; time and decision-specific

### Without capacity

- Advance directive, legal proxy, best interests; local legal and religious framework (in Egypt: family-centred decisions, consult hospital ethics/legal)

### Other topics

- Brain death diagnosis, organ donation, confidentiality, research consent (deferred), resource allocation, triage

**Key point:** State that practice follows local law and institutional policy.

# 6. Palliative care in ICU

### Integrate early

- Symptom assessment, goals-of-care conversation within 72 h of admission in high-risk patients
- Triggers: advanced dementia, MOF, prolonged ventilation, poor functional baseline

### After death

- Bereavement support, debrief staff, moral distress, Ethicus data on variation

**Key point:** Palliative care is active treatment, not abandonment.